DEPARTMENT OF HEALTH &. HUMAN SERVICES

Public Health Service

Indian Health Service
Phoenix Indian Medical Center
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Phoenix, Arizona 85016

Policy Review Statement
Reviewer: Kevin D. Turnage, MSN, RN – Policy and Procedure Officer, Nurse Consultant
Date: June 9, 2025
Policy Title: Asbestos Awareness and Control
PolicyStat ID: 18233131
Effective Date: June 2, 2025
Next Scheduled Review: June 1, 2028

Purpose of Review

The purpose of this review is to assess the *Asbestos Awareness and Control* policy for compliance with federal regulations, environmental health standards, training adequacy, and procedural clarity. Additionally, the review examines how well the policy supports safe operations for employees at Phoenix Indian Medical Center (PIMC), especially those in contact with asbestos-containing materials (ACM).

Evaluation Summary

✅ Strengths Identified

- **Regulatory compliance** is explicitly stated with adherence to:
  - 29 CFR 1910.1001 (OSHA General Industry)
  - 29 CFR 1926.1101 (OSHA Construction)
  - 40 CFR Part 61, Subpart M (EPA NESHAP)
- **Clear categorization** of asbestos work classes (I–IV) aligned with OSHA.
- **Defined training requirements** for Class III and IV asbestos workers.
- **Designated authority structure**, including the Competent Person and notification procedures to Environmental Health & Safety (EHS).
- **Description of engineering controls** and personal protective equipment (PPE) for Class III/IV activities.

⚠️ Issues Identified and Recommendations

1. Missing Citation of Current Best Practices or Guidance

- No reference to **NIOSH, EPA's AHERA standards**, or state/local public health guidance.
- **No inclusion of CDC/NIOSH recommendations** for worker health monitoring or exposure follow-up.

  Recommendation: Supplement federal regulatory citations with:

- NIOSH Pub. No. 2011–159: *Asbestos: Worker and Employer Guide*
- EPA's *Green Book* or *Managing Asbestos in Place (1990)*
- Consider including OSHA technical manuals or AHERA standards for context in K–12 or healthcare institutions.

2. No Detailed Exposure Risk Assessment Procedures

- The policy mentions exposure response through I-STAR but lacks formal guidance for:
  - Employee **exposure threshold documentation**
  - **Medical surveillance or respiratory protection clearance** procedures
  - **Air monitoring protocol**

  Recommendation:

  - Add a subsection outlining **air sampling requirements**, exposure thresholds (e.g., 0.1 f/cc TWA), and medical surveillance thresholds per 29 CFR 1910.1001.
  - Define whether a **baseline and periodic medical exam program** exists for authorized Class III/IV workers.

3. Inconsistent Training Oversight and Documentation Language

- Policy indicates training records are to be stored in the "7B folder" and via RELIAS, but this may lack clarity for new staff or reviewers.

  Recommendation:

  - Standardize language around where records are retained (e.g., "employee health file" or centralized LMS).
  - Add reference to **training verification during onboarding or prior to assignment to potential ACM contact areas**.

4. Ambiguity Around Certification Requirements

- The policy defines who can perform certain classes of work, but it does not explain:
  - What credentials or agencies certify contractors or internal staff
  - How certification status is verified and documented

  Recommendation:

  - Define which certifying bodies (e.g., EPA-accredited, state-licensed) are acceptable.
  - Assign responsibility for **tracking contractor and employee certifications** (e.g., Engineering or EHS oversight).

5. No Policy on Notifications to External Authorities

- While internal notifications to EHS are defined, there is no mention of:
  - **Regulatory reporting to EPA or OSHA** in the event of accidental release or worker exposure

- **Community/environmental disclosure** in case of extensive ACM release or remediation

Recommendation:

- Include a provision describing reporting protocol under NESHAP Subpart M and OSHA standards.
- Clarify thresholds for when external reporting is required.

6. Lack of Visual Aids or ACM Location Resources

- The policy refers to location maps stored on the Engineering share drive but does not describe:
  - Whether departments are given **access or briefings**
  - If maps are updated **annually or post-remediation**

Recommendation:

- Describe update frequency and how frontline staff can **request or review maps**.
- Encourage use of **posted ACM notification signage** in high-risk areas.

✅ Minor Improvement Opportunities

- Use plain language to define **technical terms** for non-engineering staff.
- Add **flowchart or table** summarizing PPE and work controls by asbestos work class.
- Define process for **contractor oversight**, including who ensures third-party compliance during Class I/II projects.

📌 Summary of Recommendations

Category	Issue	Recommendation
Regulatory Scope	Limited to CFR citations	Add NIOSH/EPA technical guidance
Exposure Monitoring	No air sampling/medical follow-up details	Define air monitoring, surveillance, and PPE clearance
Training	Unclear storage/verification	Standardize and align with LMS and HR compliance
Certification	No agency or oversight info	Identify approved certifiers and tracking method
Reporting	No external authority protocol	Add OSHA/EPA reporting pathway for release/exposure
Communication	ACM map access/process vague	Clarify map review/update process and signage expectations

Conclusion and Action Plan

The *Asbestos Awareness and Control* policy at Phoenix Service Unit meets minimum federal regulatory standards and provides foundational guidance for safe operations. However, it would benefit from expanded scope, clarified processes, and specific performance oversight mechanisms to ensure robust compliance, especially in the areas of **worker health protection**, **training accountability**, and **external regulatory coordination**.

I recommend the policy remain active but be revised to address the findings detailed above. These updates will enhance transparency, reduce liability, and further align the policy with national safety benchmarks and accreditation expectations.

Respectfully submitted,

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